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 / Practice Exam - 1

Correct Answer

Next Question

Practice Exam - 1

Question 6 of 117



Case: A 63 year-old female with a known history of non-ischemic cardiomyopathy, s/p ICD, type 2 diabetes mellitus, and hypothyroidism was transferred to a tertiary medical center on milrinone at 0.25 mcg/kg/min for advanced therapies evaluation, found to be hypotensive with end organ dysfunction, SCAI shock classification C, is now s/p right heart catheterization and Impella 5.5 placement. She is day 5 post-procedure, you are called by the Registered Nurse (RN) because of a change on the console.

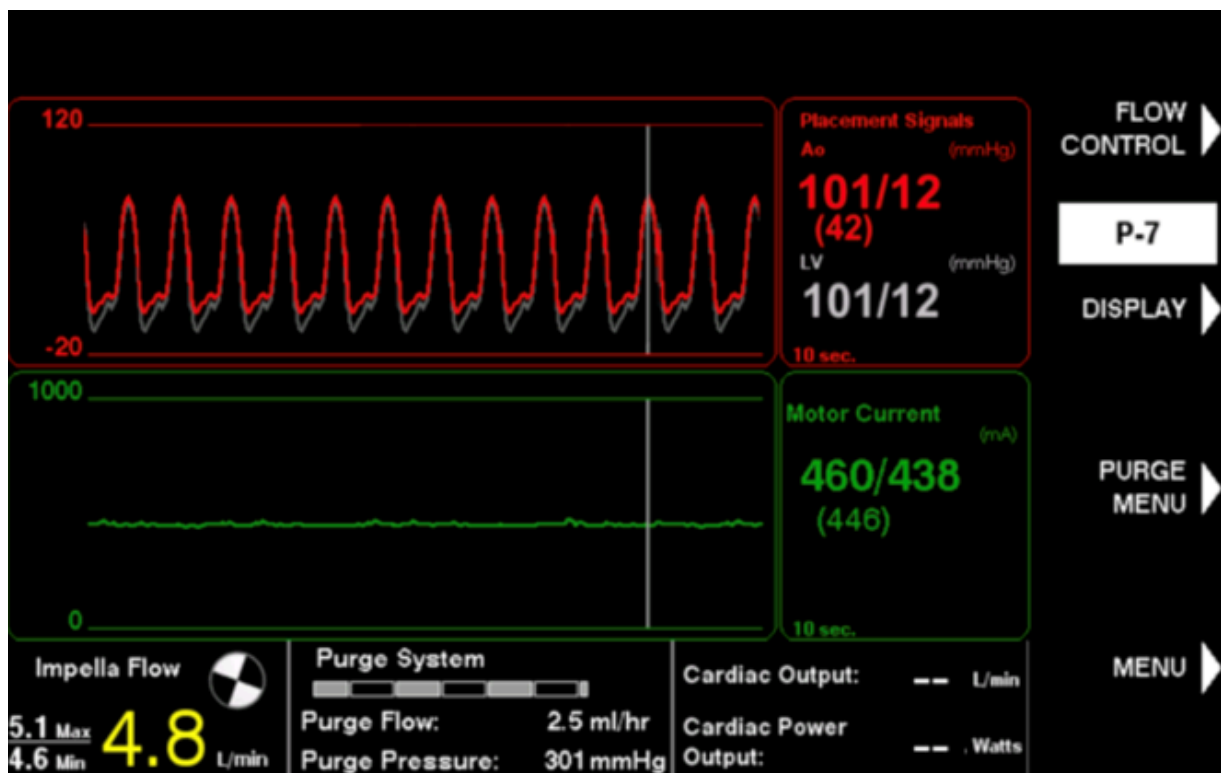


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Which of the following is the most appropriate next step?

A. You suspect low purge flow and ask the RN to change the purge cassette.

B. You suspect the Impella is deeply seated in the LV and do a bedside echocardiogram to assess positioning.

C. You suspect the patient had a stroke and take her for an urgent CT head.

D. You suspect the Impella position is in the aorta and do a bedside echocardiogram to assess positioning.

Commentary

References

Teaching Point: AHFTC needs to know proper troubleshooting for the Impella 5.5 device based on tracing seen on console.

Correct Answer: B. You suspect the Impella is deeply seated in the LV and do a bedside echocardiogram to assess positioning.

Rationale: The Ao and LV tracings are more than overlapping on this console. There is a ventricular placement signal waveform in the place of Ao signal, meaning the Ao tracing mimics the LV. Note, the motor current is also flat.

Incorrect Answers: A. The purge flow (normally between 2-30 mL/hr) and purge pressure (normally between 300-1100) are within the ranges and not low. C. There is no evidence of the patient having a stroke in this scenario. D. The LV and Ao waveforms are both arterial, therefore most likely Impella position is in the aorta.

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